Which treatment this guide covers This guide covers lorlatinib alone for ALK-positive advanced non-small-cell lung cancer. The available Irish National Cancer Control Programme (NCCP) v3 document lists use after particular earlier ALK medicines. The current European licence also includes people who have not had an ALK inhibitor. The team must confirm your treatment setting and the current Irish protocol and access arrangements before prescribing; the source versions currently disagree.

NCCP 00570 · VERSION 3

Lorlatinib therapy

Lorlatinib is taken once daily continuously, with no planned rest week. Treatment continues while it is helping and side effects remain acceptable; there is no fixed number of treatment cycles.

Make treatment fit into daily life

The usual starting dose is 100 mg once daily; use the dose on your prescription. Take it at about the same time each day, with or without food. Swallow whole; do not crush, chew or split tablets, or use damaged tablets. Avoid grapefruit and grapefruit juice.

Before starting

Agree prescription supply, baseline tests, the source/access review and the 24-hour oncology contact plan.

Taking treatment at home

The usual starting dose is 100 mg once daily; use the dose on your prescription. Take it at about the same time each day, with or without food. Swallow whole; do not crush, chew or split tablets, or use damaged tablets. Avoid grapefruit and grapefruit juice. If a dose is missed, take it when remembered if the next dose is at least four hours away; otherwise skip it. Do not take two doses together. If you vomit after a dose, do not take another tablet; take the next dose at the usual time. These instructions follow the current product leaflet.

Appointments and refills

Keep enough tablets for the prescribed plan. Agree which team books each test and reviews results. Do not restart after a hold without instructions.

Support for daily life

Bring a medicines list and symptom notes. Ask about help with travel, work, memory or mood changes; involve someone you trust if useful.

When treatment finishes

Lorlatinib may harm an unborn baby and make hormonal contraception less effective. Women who could become pregnant need highly effective non-hormonal contraception during treatment and for at least 35 days afterwards; discuss any unavoidable hormonal method and barrier protection. Men with partners who could become pregnant need effective contraception including a condom during treatment and for at least 14 weeks afterwards; condoms are also needed with a pregnant partner. Discuss fertility preservation before treatment. Do not breastfeed during treatment; agree when it can safely resume with the team.

Who arranges each check?

Agree the due window, booking team, result reviewer and whom to contact if an appointment or result is missing. This reference page does not book or record a test.

Sources and review

The NCCP regimen supplies the treatment rules. Separate guidelines supply the longer-term monitoring recommendations, with their qualifications retained.

- NCCP 00570 v3 · official regimen
- EMA Lorviqua current product information and patient leaflet
- NCCP NCIS antiemetic medicines V8 (parenteral scope; no oral schedule copied)
- CROWN 7-year update, Annals of Oncology 2026: first-line lorlatinib versus crizotinib
- Solomon et al., Lancet Oncology 2018: global phase 2 ALK cohorts

Source reconciled; independent clinical, pharmacy and patient/PPI review pending